

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

# Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

## Part One — What Real Accessibility and Inclusion Actually Look Like

### Section 1.6 — The Triage Question

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Sections 1.3 through 1.5 examined Alberta's response to disability that arrives early — through congenital, developmental, or childhood-acquired conditions — and the lifecycle failures that response produces. Section 1.6 closes Part One by examining the disability category Alberta responds to differently. The category is acquired disability through workplace injury. The response is the architecture administered by the Workers' Compensation Board of Alberta (WCB-Alberta) under the *Workers' Compensation Act*. The architecture is, on its own terms, the most coherent triage response Alberta has built for any disability population.

The contrast is the question this section asks. Why does the province respond to one disability route with full triage, full rehabilitation, full income protection, full continuity of medical care, full vocational support, and statutory protection from administrative re-litigation — while responding to disability that arrives through any other route with the architecture documented in Sections 1.3 through 1.5? The answer is not clinical. The clinical needs of a worker with a permanent acquired disability are not different in kind from the clinical needs of an adult with a permanent congenital or developmental disability. The answer is political. Alberta has chosen, through legislation and program design, to build full architecture for one route and scrutiny architecture for all others. That choice is what Section 1.6 documents, and what the blueprint Part Three exists to reverse.

## The Historic Compromise

The Workers' Compensation Board of Alberta is the operational descendant of a Canadian legal architecture that is more than a century old. The architecture is built on the Meredith Principles, established in the 1913 Meredith Report tabled in the Ontario Legislature by Sir William Meredith, an Ontario lawyer, politician, and judge appointed in 1910 to study workers' compensation systems internationally. The report's foundational argument was that the existing legal regime — under which an injured worker could pursue compensation only by suing the employer in court, with all the cost, delay, and adversarial uncertainty that litigation entails — was inadequate to provide just compensation, harmful to both workers and employers, and unsustainable as social policy in an industrializing economy.

The Meredith Report proposed a *historic compromise*. Workers would relinquish their right to sue their employers in court for workplace injury. In exchange, employers would fund a collective insurance system that would provide injured workers with no-fault, prompt, and secure compensation regardless of who was at fault for the injury. The compromise rested on five interlocking principles, each of which remains embedded in Canadian workers' compensation law today:

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*No-fault compensation.* Injured workers receive benefits regardless of how the injury occurred. Fault is irrelevant to entitlement. The worker does not have to prove negligence. The employer does not have to defend negligence. The compensation is the focus.

*Security of benefits.* A statutory fund is established to guarantee that compensation will be paid. Workers do not face the risk that an employer's bankruptcy, dissolution, or refusal to pay will leave them without recourse. The fund exists, the obligation exists, the benefits are paid.

*Collective liability.* All covered employers contribute to the common fund. The cost of workplace injury is distributed across the entire employer base, not borne individually by the employer of the specific injured worker. The system is, in structural terms, a publicly mandated insurance regime funded by employers and administered for the benefit of workers.

*Independent administration.* The Workers' Compensation Board operates at arms-length from government, with statutory independence designed to insulate compensation decisions from political pressure. Ministerial powers over the Board are limited by design. The point of the limitation is to prevent the compensation regime from being narrowed, defunded, or restructured for political reasons that have nothing to do with the welfare of injured workers.

*Exclusive jurisdiction.* Workers' compensation is the only mechanism through which workplace injury claims may be resolved. Workers cannot sue. The Board's decisions are final, subject to internal appeal and limited judicial review. The exclusivity is the price of the bargain. It is also the protection that secures the bargain.

The Meredith Principles were enacted in Ontario's 1914 *Workmen's Compensation Act*, proclaimed in 1915. Other Canadian provinces followed. Alberta's first workers' compensation legislation was passed in 1918. All ten provincial and three territorial workers' compensation systems in Canada trace their statutory foundations to the Meredith Report. The legislation has been amended many times across the intervening century, but the core architecture has held. The reason it has held is that the bargain works for both parties. Workers receive prompt, secure, no-fault compensation. Employers receive predictable, collectivized cost and protection from open-ended tort liability. The province receives a functional disability response that does not require it to administer the underlying litigation system the bargain replaces.

## What WCB-Alberta Currently Provides

The WCB-Alberta architecture, in its current operational form, is the working example of a triage-mode disability response Alberta is capable of building when it chooses to. The following is what the program provides to a worker injured on the job today.

**Coverage begins immediately.** The *Workers' Compensation Act* provides no waiting period before coverage takes effect. A worker injured on Monday is covered on Monday. There is no application backlog comparable to the FSCD wait list documented in Section 1.3 or the PDD wait list documented in Section 1.4. The architecture is designed for the moment the injury happens, not for the moment the bureaucratic process has caught up to the injury.

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**Compensation benefits are not taxable.** Income replacement under WCB is paid at up to 90 percent of net pre-accident earnings. The benefits are paid directly to the worker (or, in some cases, through the employer who continues paying the worker and is then reimbursed by WCB). The non-taxable status of the benefits is statutorily protected. The income replacement is calibrated to the worker's pre-accident standard of living, not to a province-wide subsistence floor.

**A care plan is developed jointly with the worker, employer, and healthcare providers.** The care plan progresses through documented phases — entitlement determined, care plan defined, treatment and rehabilitation, return-to-work planning, and ongoing review. The plan is the product of collaboration between the worker, the worker's physicians, the employer, and the WCB caseworker. It is not produced by an adjudicator working from a paper file alone. It is not produced by a regional review committee operating without the worker's involvement. The worker is, by program design, a participant in their own care.

**A claim is assigned to an adjudicator who may transfer the claim to a case manager for rehabilitation support.** The case-management model provides continuity of relationship between the worker and the WCB staff person responsible for navigating the claim. The case manager is a known individual. The worker is not navigating a phone tree. The worker is not waiting nine months between calls to discover whether their file has moved.

**Occupational Injury Service (OIS) provides access to physicians experienced in work-related injuries.** WCB has built a clinical network specifically designed for the population it serves. The injured worker is not navigating a generalist primary-care system that does not have the specialty knowledge their injury requires, nor paying out of pocket for specialist assessments the public system has not made available. The clinical infrastructure is part of the program.

**Medical benefits cover all costs of medical care related to the compensable injury.** This includes physician visits, diagnostic imaging, specialist consultations, prescription medications, physiotherapy, occupational therapy, psychology, surgery where indicated, and other treatment costs the worker would otherwise bear or wait for through the public healthcare system. Alberta Health Care basic coverage is the worker's responsibility for non-injury healthcare; everything related to the injury is covered by WCB. The injured worker is not facing pharmacy denials, formulary exclusions, or out-of-pocket costs for the treatment of the condition WCB has accepted.

**Modified work programs allow injured workers to return to work at the earliest safe opportunity at pre-accident pay rate.** The program is designed to maintain the worker's connection to employment, income, and meaningful work during recovery. The employer plays an active role; WCB provides incentives to employers (lower premiums) for offering modified work successfully. The architecture is collaborative, not adversarial.

**Re-employment services and vocational training are available where the worker cannot return to the pre-accident job.** If recovery does not permit a return to the original work, WCB provides retraining, skills development, and re-employment services to support the worker's transition to alternative employment. The program does not, in this scenario, simply terminate income support and leave the worker to navigate the open labour market unassisted. The vocational architecture is part of the program.

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**Reimbursement is available for vehicle modification, home modification, personal care, translation and interpretation services, and other accommodation costs.** Where the injury produces a permanent disability requiring assistive infrastructure, the program provides the infrastructure. The worker is not paying privately for wheelchair-accessible vehicle modifications, home renovations, or attendant care that the injury makes necessary. The program recognizes that disability requires architecture beyond income, and it provides the architecture.

**The injured worker is not required to re-prove the injury every six months.** Once entitlement is determined, the worker's claim status persists until a documented change in clinical status warrants review. The worker is not navigating annual reassessments, periodic re-applications, or the kind of administrative re-litigation documented in Sections 1.4 and 1.5 as the AISH and PDD experience. The continuity of entitlement is part of the bargain.

**The program operates at arms-length from political administration.** Ministerial authority over WCB benefits is statutorily constrained. The minister cannot, by regulation, narrow the eligibility definition for workplace injury, or repeal the benefits indexation, or bar appeal of categories of decisions the minister finds inconvenient. The Bill 12 architecture documented in Section 1.4 has no equivalent in the WCB legislation. The political insulation is structural.

This is the architecture. It is operational. It exists today, in Alberta, administered by a Crown corporation under a provincial statute, funded by a collective employer-paid insurance fund, providing the supports above to approximately 150,000 active claims at any given time. The province has built it. The province administers it. The province does not contest its existence.

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#### WHAT WCB-ALBERTA CURRENTLY DELIVERS

**No waiting period.** Coverage begins on the day of injury. No three-year wait list.

**Up to 90 percent of net pre-accident earnings, non-taxable.** Income calibrated to standard of living, not subsistence floor.

**Care plan developed jointly with the worker, employer, and physicians.** Worker is a participant in their own care by design.

**Continuity of caseworker.** Adjudicator and/or case manager assigned to the claim — a known individual, not a phone tree.

**Occupational Injury Service.** Clinical infrastructure built for the population served, not outsourced to a general healthcare system without capacity.

**All injury-related medical care covered.** No out-of-pocket costs, pharmacy denials, or formulary exclusions for the condition WCB has accepted.

**Modified work, vocational retraining, re-employment services.** The program provides architecture beyond income.

**Vehicle modification, home modification, personal care, translation / interpretation reimbursed.** Disability requires architecture beyond income; the program provides it.

**No requirement to re-prove the injury every six months.** Continuity of entitlement until clinical status changes.

**Operates at arms-length under the Meredith principle of independent administration.** Statutory protection from political restructuring.

**Approximately 150,000 active claims at any given time.** Operational, funded, durable, replicable.

## What the WCB Architecture Establishes

The WCB-Alberta architecture establishes, by example, that the province is capable of building a triage-mode disability response. The objections most commonly raised against extending equivalent architecture to other disability populations — that it would be too expensive, too administratively complex, too vulnerable to fraud, too disincentivizing of work, too difficult to operate at arms-length from political pressure — are answered by the WCB architecture's existence. None of those objections has prevented the WCB system from operating. None of those objections has prevented the system from delivering the supports listed above. The objections are, in the WCB context, not allowed to prevail.

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The architecture's design choices are documented and replicable. *Triage instead of adjudication backlog.* WCB processes claims on the day of injury, not three years later. *Continuity of caseworker instead of phone-tree churn.* WCB assigns a case manager whose name the worker knows. *Care plans developed with the worker instead of imposed on the worker.* WCB requires worker participation in plan development by design. *Clinical infrastructure built into the program instead of outsourced to a healthcare system without capacity.* WCB has the Occupational Injury Service. *Income replacement calibrated to pre-injury standard of living instead of subsistence floor.* WCB pays up to 90 percent of net earnings. *Continuity of entitlement instead of perpetual reassessment.* WCB does not require six-monthly re-proof. *Statutory insulation from political restructuring instead of ministerial discretion.* WCB operates at arms-length under the Meredith principle of independent administration.

Each of these design choices is a deliberate departure from the architecture documented across Sections 1.3 through 1.5. Each is administratively possible, fiscally sustainable, and politically durable. The proof of each is the program's continued operation. The province knows how to build this. The province has built it. The province continues to build it. The province has chosen not to extend it.

## The Triage Question

Imagine two Albertans. The first is a forty-year-old construction worker, employed by a covered employer, who falls from scaffolding and sustains a spinal cord injury. The second is a forty-year-old adult with autism and a co-occurring intellectual disability, eligible for AISH and on the PDD waitlist, with functional support needs comparable in clinical complexity to the construction worker's post-injury needs.

The construction worker's injury triggers, immediately, the WCB architecture documented above. A claim is opened on the day of the fall. An adjudicator is assigned. A case manager is assigned for rehabilitation support. A care plan is developed jointly with the worker, the worker's physicians, and the employer. Income replacement begins at up to 90 percent of pre-accident net earnings, non-taxable. Medical benefits cover all injury-related care, including specialist consultations, surgery, rehabilitation therapy, prescription medications, and assistive equipment. Home modifications are reimbursed. Vehicle modifications are reimbursed. Personal care is reimbursed. If the worker cannot return to pre-accident employment, vocational retraining and re-employment services are provided. The worker is not required to re-prove the injury every six months. The worker is not navigating the architecture alone; a known case manager is the point of contact. The architecture is collaborative, time-sensitive, and protected from political restructuring.

The forty-year-old adult with autism and intellectual disability faces a different architecture. The PDD application requires a Psychological Assessment Report, which the public system does not provide within the relevant window; the family pays \$2,400 privately. The AISH application requires a Part B Medical Report; the family doctor of fifteen years has retired and not been replaced; a private psychiatric assessment service completes the report at a cost of \$700. The PDD wait list, per Inclusion Alberta's documentation, averages four and a half months for an eligibility decision and an additional fourteen months for service commencement after that decision. The Outcome Plan that was supposed to be developed with the adult and the family is, in seventy percent of waitlisted cases, not developed at all. The income provided through AISH is \$1,940 per month, transitioning to \$1,740 under ADAP on July 1, 2026,



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with the transition decision unappealable. There is no equivalent of the Occupational Injury Service. There is no case manager whose name the adult knows. There is no care plan developed jointly. There are no home modifications reimbursed beyond what the FSCD program covered before the eighteenth birthday. There is no vehicle modification program. There is no statutory protection from political restructuring; the Bill 12 architecture documented in Section 1.4 is the architecture this population now navigates.

The clinical needs are, in the comparison drawn here, substantially equivalent. The functional support requirements are comparable in scope. The lifelong nature of the disability is, in both cases, established and documented. The province has accepted, in both cases, that the population requires support. The architectures the province provides for the two populations are different by orders of magnitude.

The question Section 1.6 asks is the question the comparison demands. Why? The clinical answer is that there is no clinical answer. The two populations have comparable needs. The legal answer is that the province has chosen, through legislation, to build one architecture and refuse to build the other. The political answer is that the construction worker entered the system through a route that the province treats as worthy of full triage, and the adult with autism entered the system through a route that the province treats as worthy of scrutiny. The selectivity is what is being chosen. The selectivity is what the blueprint Part Three exists to remove.

## What the Selectivity Reveals

The province's response to acquired disability through workplace injury is not better designed because workplace injury is more clinically deserving of triage than congenital or developmental disability. It is not better designed because the political constituency of injured workers is more sympathetic than the political constituency of disabled adults with autism, intellectual disability, FASD, or chronic mental illness. It is not better designed because the fiscal cost of WCB-mode architecture is somehow lower than the fiscal cost of the architecture documented in Sections 1.3 through 1.5; the fiscal cost of the under-built architecture for the latter populations is, as the chronological evidence chain in Section 1.3 establishes, larger in aggregate than the cost of building the architecture properly would have been.

The province's response to acquired disability through workplace injury is better designed because the historic compromise the WCB architecture is built on contained, at its inception, three structural protections that the architecture for other disability populations does not contain.

The first is the *no-fault principle*. WCB does not require the worker to prove that the injury happened in a way that justifies support. The worker is supported because the worker is injured. The architecture for AISH, PDD, FSCD, and ADAP, by contrast, requires the disabled person to prove, repeatedly, that the disability is severe enough, permanent enough, and disabling enough to qualify under the program's criteria. The proof requirement is itself a form of fault attribution — the disability must be demonstrated against a burden of evidence the worker is not required to meet.

The second is the *security of benefits principle*. WCB benefits are statutorily guaranteed through a fund insulated from year-by-year budget pressure. The benefits cannot be cut by the minister through regulation, indexation cannot be removed by repealing a schedule, transitions to lower-benefit replacement programs cannot be imposed without appeal. The architecture for AISH, PDD, FSCD, and

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ADAP, by contrast, is operated through annual budget allocations, ministerial discretion, and legislation amenable to amendment as documented in Bill 12. The benefits are, by structural design, exposed to the political volatility the WCB architecture was created to prevent.

The third is the *independent administration principle*. WCB operates at arms-length from the minister responsible. The architecture for AISH, PDD, FSCD, and ADAP is operated directly by the Ministry of Seniors, Community and Social Services, with the minister exercising regulatory authority over eligibility, benefit levels, appeal jurisdiction, and program structure. The Bill 12 grant of authority to the minister to limit categories of decisions appealable to the Citizens Appeal Panel is a feature the WCB architecture would not permit. The political volatility that has produced the current dismantling of the AISH architecture is, in the WCB context, statutorily prevented.

The selectivity is structural. The WCB architecture's existence proves that triage-mode disability response is possible. The absence of equivalent architecture for other disability populations proves that the absence is a choice. The choice is what Part Two examines (other Canadian provinces and the federal Accessible Canada Act made different choices) and what Parts Three through Five propose to reverse (the blueprint, the proposed legislation, and the closing argument).

## What the Closing Should Say

The campaign's working framing — *Do we deny rehabilitation? No. Do we refuse WCB? No. Do we welcome them into the community when they are most vulnerable? Yes. The government should be the first to triage, not the community* — captures the core of Section 1.6's argument and the spirit of the blueprint Part Three is built to deliver. The argument is not that workplace injury should be supported less, or that WCB should be narrowed, or that the architecture documented above should be dismantled in the name of equity. The argument is the opposite. The argument is that the architecture WCB demonstrates is the right architecture, that it should be the model rather than the exception, and that the province's failure to extend equivalent architecture to other disability populations is a failure of will, not a failure of capacity.

A fair province does not choose which disabled citizens deserve triage. A fair province builds triage as the default response to disability of any origin. The route through which a person becomes disabled — workplace injury, motor-vehicle collision, congenital condition, developmental disability, progressive disease, mental illness, age-related decline, sudden acquired condition outside the workplace, accumulated chronic illness, environmental harm — is not a clinical fact. It is a legal fact. The clinical facts are functional impairment, support need, and the duration of the condition. The clinical facts do not vary by origin route. The architecture should not vary by origin route either.

The blueprint Part Three builds will require, at minimum, the following triage-related commitments: a statutory recognition that disability of any origin is entitled to a coordinated provincial response equivalent in scope to the WCB architecture; the establishment of integrated case-management infrastructure across all major disability programs (FSCD, PDD, AISH, ADAP, healthcare, housing) so that disabled Albertans are not navigating multiple ministries through multiple application processes for support that should be coordinated by the province; the restoration of independent administration through the full reinstatement of the Citizens Appeal Panel's jurisdiction across all categories of disability-program decisions; statutory benefits indexation protected from regulatory rollback; clinical infrastructure equivalent to the Occupational



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Injury Service for the disability populations the public health system currently fails to serve; and a binding statutory commitment that the province will not, in any future legislation, narrow the disability response architecture below the floor that this blueprint establishes.

The legislation Part Three proposes is the architecture that makes those commitments enforceable. The argument Part Four makes is that the current selectivity — full triage for one disability route, scrutiny architecture for all others — is a deliberate political choice made by ministers with documented biographical proximity to the populations the choice is harming. The closing argument Part Five makes is that the architecture being defended in this document is the architecture every Albertan will eventually need, in some mode, through some route none of us can predict. The triage question is not, in the long run, a question about disabled people. It is a question about what the province will do for any of us when our turn comes.

## What the Comparator Looks Like

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

### ILLUSTRATION — TWO EMERGENCY ROOMS, ONE PROVINCE

*Imagine two emergency rooms, in two adjacent communities, in the same province, on the same day.*

*In the first emergency room, a forty-year-old construction worker arrives by ambulance after falling from scaffolding. The intake nurse confirms the injury occurred at work. A WCB claim number is generated within hours. The patient is admitted, assessed, stabilized, and scheduled for surgery. By the end of the week, an adjudicator has been assigned. By the end of the month, a case manager is in regular contact with the patient and the patient's spouse. By the end of the third month, a care plan is in place that includes physical rehabilitation three times per week, prescription coverage, mileage reimbursement to and from appointments, and a return-to-work assessment scheduled for the six-month mark. The patient's income is being replaced at 90 percent of pre-accident net earnings, non-taxable, delivered by direct deposit on a regular schedule. The home is being assessed for accessibility modifications. A vehicle modification application has been initiated. The patient does not, at any point in this process, pay out of pocket for any of it. The patient does not, at any point in this process, fill out a 27-page reassessment form. The architecture moves around the patient, on the patient's clinical timeline, with continuity of caseworker and continuity of medical relationship.*

*In the second emergency room, a forty-year-old adult with autism and a co-occurring intellectual disability arrives in crisis. The patient's primary caregiver, the patient's mother, died three weeks ago. The patient has been living alone since then. The patient has not eaten consistently. The patient has not slept. The patient's medications, which the mother managed, have been taken inconsistently. The patient is admitted, assessed, stabilized, and held for psychiatric observation. The hospital social worker contacts PDD. PDD confirms the patient is on the waitlist. PDD confirms the patient has been on the waitlist for fourteen years. PDD confirms that under the urgent and critical needs threshold, the death of the primary caregiver and the resulting crisis presentation may now meet the threshold for support. The hospital cannot discharge the patient because no community placement is available. The patient remains in the hospital for forty-three days, occupying a bed that cost the provincial healthcare system several thousand dollars per day. The patient is eventually placed in a long-term care facility designed for elderly residents, where the patient will live, in a clinical environment unsuited to the patient's actual needs, until further architecture becomes available, at the province's eventual expense.*

*The construction worker and the adult with autism are equally Albertans. Their disabilities are equally permanent. Their support needs are equally documented. The architectures the province provides for them are not equal. The inequality is not clinical. The inequality is the structure of Alberta's current disability law. The blueprint this document builds is the proposal to remove the inequality and build, for both, the architecture either of them would, in a fair province, be entitled to.*

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## What This Section Establishes, and What Part One Closes With

Section 1.6 has documented the comparator the rest of Part One has been building toward. The Workers' Compensation Board of Alberta administers, today, a triage-mode disability response that demonstrates the province's structural capacity to build proper architecture for a disabled population. The architecture's design features — no-fault entitlement, secure benefits, independent administration, clinical infrastructure, continuity of caseworker, integrated care planning, vocational support, accommodation reimbursement, and statutory protection from political restructuring — are documented, operational, and replicable. The province has built the architecture. The province administers the architecture. The province has chosen not to extend the architecture to the disability populations Sections 1.3 through 1.5 examined.

The selectivity is structural, deliberate, and reversible. The blueprint Part Three builds is the architecture that reverses it. The legislation Part Three proposes is the binding mechanism that protects the new architecture from being narrowed back to the current state once the political winds change. The argument Part Four makes is that the current selectivity is a choice made by named ministers with documented biographical proximity to the populations the choice harms. The closing argument Part Five makes is that the architecture every Albertan will eventually need, through some route none of us can predict, is the architecture this blueprint exists to build.

The lifecycle frame established in Section 1.2 holds across Sections 1.3 through 1.6. Childhood. Transition and adulthood. Caregivers and the family safety net. Acquired disability through routes the province has chosen to support, and acquired disability through routes the province has chosen not to support equivalently. The pattern is consistent across every life stage and every disability category. The architecture's failures are not random; they are structural. The architecture's selectivity is not accidental; it is legislated.

Part Two follows this conclusion into the comparator question across jurisdictions. The federal *Accessible Canada Act*, Ontario's *Accessibility for Ontarians with Disabilities Act*, Manitoba's *Accessibility for Manitobans Act*, Nova Scotia's *Accessibility Act*, British Columbia's *Accessible British Columbia Act*, Saskatchewan's accessibility legislation, the United Nations *Convention on the Rights of Persons with Disabilities*, and the documented architectures other Canadian provinces and the federal government have built are all, in their respective scopes, the legal frameworks Alberta has chosen not to build. Part Two examines what those frameworks contain, what they have produced, and what Alberta's deliberate non-construction of equivalent architecture reveals about the choice the province has made.

Part One has documented what is failing. Part Two documents what other jurisdictions have built. Part Three proposes what Alberta should build. Part Four assigns the responsibility for the current direction. Part Five closes on the principle the entire document is built to defend: a fair province builds the floor, not the ceiling, for everyone.

The triage question Section 1.6 raised is the question the entire blueprint exists to answer. The answer is that disabled Albertans are entitled, under any plausible standard of fairness, to architecture equivalent to what the province has built for the populations it has chosen to triage. The route through which a person becomes disabled is not a justification for differential treatment. The route is, at most, an explanation of how the architecture's gaps came to exist. The blueprint the rest of this document builds is the proposal to

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close those gaps, statutorily, durably, and for everyone.

That is the work Part Two through Part Five completes.

That is the work this document exists to make unavoidable.

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**Part One closes here.** The lifecycle frame has held. The evidence chain has been established. The comparator has been named. The blueprint begins.